



STRATEGIC OPERATIONAL REQUEST

TO: Healthcare Data Analytics Team 4

FROM: Chief Operations Officer, NovaCare Hospital

DATE: March 12, 2026

SUBJECT: URGENT INVESTIGATION INTO OPERATIONAL "STIFFNESS"

Our leadership team is increasingly concerned by recurring reports of operational gridlock across several key wards. While our baseline volume metrics appear stable, frontline staff are reporting a palpable sense of "chaos" that our current reporting tools fail to explain.

THE CORE COMPLAINT

"We are constantly diverting ambulances and declaring bed shortages, yet our occupancy dashboards show open beds. Patients seem to be getting 'stuck' in our system, particularly during transition points. We feel like we are operating at 120% capacity even when the numbers say 80%."

KEY PRESSURE POINTS FOR INVESTIGATION

- **The 6:00 AM Shift Meltdown:** Incoming day shifts are reporting a massive backlog of unprocessed admissions from the night before. We need to determine if this is a genuine arrival surge or a documentation failure.
- **The Pediatric Ward Resource Paradox:** Pediatric department heads are reporting severe overcrowding, but our actual pediatric admission numbers remain stable. We suspect our pediatric beds are being used for non-pediatric needs, but we lack proof.

- **The Sunday-Monday Capacity Crisis:** Every Monday morning we face a total system stall. We suspect a "weekend effect" where patients accumulate before formal admission, but we need to see exactly when and why this pressure peaks.
- **The Discharge "Trap":** Clinicians claim they are ready to move patients, but the beds remain occupied. We suspect a massive bottleneck at the post-acute care transition, specifically for rehabilitation placements.

EXPECTED ANALYTIC OUTPUT

We require Team 4 to stop providing anecdotes and start providing **evidence-based diagnostics**. Your task is to identify where the flow is actually breaking and determine if our operational bottlenecks are clinical or administrative in nature.